PractixOS

The practice operating system for India's independent clinics.

SEED · INDIA
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CONTACT

Clinic software in India is sold on features and priced per doctor. PractixOS is sold on **a number the clinic can audit**: the rupees it recovered this month from appointments that would not have happened. The subscription is a rounding error against it.

THE PROBLEM

India has **428,668** outpatient practitioners. The overwhelming majority run a paper register, a WhatsApp thread and an Excel sheet, and they leak money in five predictable places every week.

Urban no-show rates routinely run **20–30%**. Phased treatment plans are silently abandoned after visit two. Lab remakes are paid for by the dentist. Consumables used chairside never reach the invoice. None of it is visible, so none of it is fixed.

What is sold to them makes it worse: per-seat pricing that pushes staff onto shared logins, or a directory aggregator that rents their own patients back to them beside a competitor's sponsored listing.

THE PRODUCT

One mobile-first application, in five Indian languages, covering the front desk, the chair, the nurse's slate, billing and UPI, ABDM compliance and the owner's P&L — built for the phone each of those people is already holding.

The part that is not a feature list is the **Recovered Revenue ledger**. Every rupee the platform saves is written as one evidenced row, pointing at a delivery receipt, a payment reference or a work order. It is append-only and it refuses to double-count.

This is also the churn defence. A subsidy keeps paying after a clinic cancels. Recovered revenue stops. The ledger is a monthly, evidenced statement of exactly what ends the day they leave.

THE UNIT THAT SELLS IT — TWO-CHAIR DENTAL PRACTICE, 400 APPOINTMENTS A MONTH

LEAK, AND WHAT CLOSES IT	VOL.	UNIT	RECOVERED
Patient forgets — confirmed WhatsApp reminder in their language `no_show_prevented`	5	₹800	₹4,000
High-value slot booked casually — UPI reservation deposit `deposit_converted`	2	₹2,500	₹5,000
Plan abandoned after visit two — clinical recall engine `plan_resumed`	1	₹6,000	₹6,000
Lab remake — work order that cannot be submitted incomplete `remake_avoided`	0.5	₹1,400	₹700
Consumable never invoiced — chairside notes carry onto the bill `consumable_billed`	15	₹60	₹900
Recovered per month, against a ₹1,999 subscription			**₹16,600**

These are deliberately conservative. The headline is not the total, which varies by practice — it is the break-even: **the subscription has paid for itself by the third recovered no-show of the month.**

BUSINESS MODEL

₹999/mo	₹1,999/mo	₹3,499/mo
SOLO PRACTICE	CLINIC PRO	AI ENTERPRISE

- **Flat per facility, never per seat.** Unlimited front-desk and nurse logins. Per-seat pricing is what drives Indian clinics onto shared accounts and destroys the audit trail we are legally required to keep.
- **88–92% gross margin.** Marginal delivery cost is ₹100–250 per clinic per month, all in: WhatsApp utility messages, gateway fees and support.
- **Self-service onboarding and CSV import ship in v1.** At ₹999 the unit economics do not permit a sales call, so the product has to close itself.
- **The ledger is the pricing instrument.** We capture roughly 12% of the value we prove. New cohorts price against twelve months of audited recovery, not a feature comparison — which is what takes the market from ₹356 crore to ₹800 crore.

WHY NOW

- **ABDM v3 API conformance becomes mandatory in July 2026.** Every incumbent has to re-certify. The switching window opens for everybody at once.
- **NHA Corrigendum 7, April 2026,** cut the digital-health incentive for an outpatient record from ₹20 to ₹5, excluded the first 100 transactions a month and restricted it to KYC-verified ABHA. Every competitor selling "the government pays for your software" has lost its pitch overnight. We rebuilt the ROI case off the subsidy deliberately, and we can show the arithmetic.
- **UPI and WhatsApp are now universal in the waiting room,** which is what makes deposits and reminders cost almost nothing to deliver.

MARKET

We size in **facilities, not practitioners**, because we price per facility: **266,000** solo and small-polyclinic facilities nationally, from a base of 428,668 practitioners.

₹150 cr **Beachhead — India dentistry.** ~50,000 facilities. Highest procedure value, a natural recall cycle, and the only vertical that comes with a distribution channel. **10% share = ₹15 cr ARR.**

₹356 cr **India, four verticals, at today's list price.** All 266,000 facilities, at the ₹999–1,999 we charge now. This is the floor, and it is the number we plan against.

₹800 cr **The same base, priced to the value we prove.** ₹999 is under 1% of a clinic's collections, against 2–4% in every comparable market. The ledger is what earns the re-rate.

The honest caveat, before you raise it. This is subscription revenue only. UPI person-to-merchant is zero-MDR by regulation, so the payments take-rate that carries Toast and Shopify is simply not available in India — and we are not sizing an adjacent line we have not built.

WHY IT IS HARD TO COPY

A new specialty is configuration, not a rebuild. Queue stages, recall rules, chairside measurements, packages and inventory are rows in a table. A continuous- integration gate fails the build if any line of code branches on a clinic's specialty. We added a complete ophthalmology practice — dilation stage, visual acuity, IOP, optical dispensing — without writing a line of logic. A horizontal competitor forks its codebase to do the same thing; a single-vertical competitor cannot do it at all.

- **The ledger compounds.** Twelve months of evidenced recovery is a switching cost no feature comparison beats.
- **Privacy enforced by the database, not the application.** Tenant isolation at the row level; a cross-tenant read returns zero rows. A zero-knowledge psychotherapy vault, with no administrator override, follows in Phase 2.
- **Cost is architectural.** Entirely free and open source on one VM inside India: ₹0 a month of infrastructure at pilot scale. We can price where a field sales force cannot follow.

DISTRIBUTION

The **dental laboratory** is the cheapest channel in the market and nobody is using it. One commercial lab serves 100–300 dentists, the relationship is already commercial, and our structured work order — tooth number, shade, due date, un-submittable if incomplete — reduces the lab's own remake rate. The lab has a reason to introduce us. This gets tested before any sales hire.

WHERE WE ARE

Specification v2.2 is complete and the end-to-end product is prototyped, not sketched. It runs four live workspaces, including the ophthalmology practice that proves the configuration thesis, and it is verified on every commit.

`786` logic assertions · `630` layout screens, 7 widths, zero overflow · `4` isolated tenants · `5` Indian languages · `13` spec sections

THE ASK

Raising `₹ __ cr` to take dentistry from prototype to `__` paying clinics over 18 months, and to land the first follow-on vertical on the same engine. Funds: a founding engineering pair through ABDM v3 certification; a clinical product lead embedded in pilot practices in Bengaluru, Pune and Hyderabad; the dental-laboratory channel test. Infrastructure is not a line item until roughly 300 clinics.

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